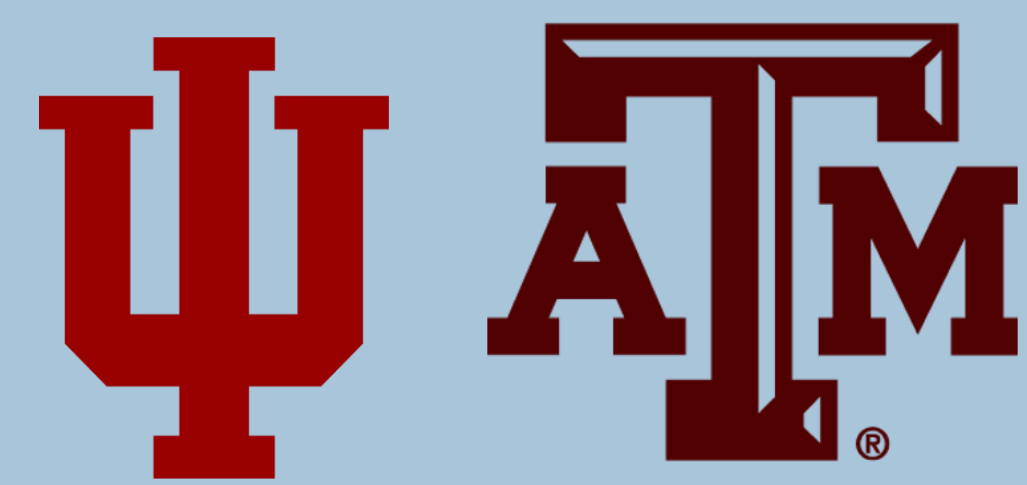


Exploring Health Disparities and Social Support in Type 2 Diabetes Management Among Black/African American Men: A Dual-Framework Qualitative Analysis

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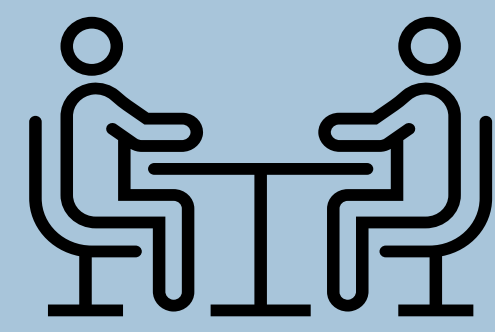


Introduction

- This study aimed to explore how cultural, structural, and interpersonal factors influence diabetes self-management among Black/African American men with type 2 diabetes (T2D).
- To capture these dynamics, this study utilized two frameworks: the Health Disparities and Discrimination Framework (HDDF) and House’s Social Support Theory (SST).
- Together, these perspectives highlight how discrimination, inequities, and social support shape men’s T2D management experiences.

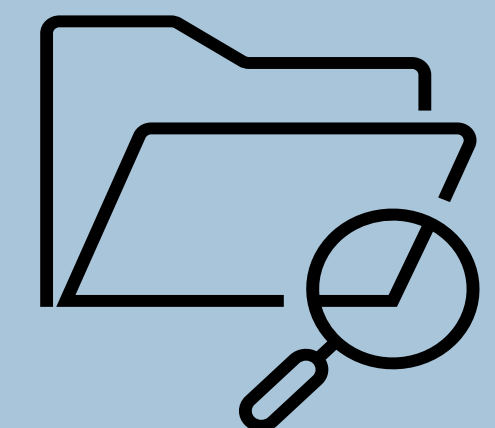
Methods

Semi-structured qualitative interviews were conducted with 60 Black/African American men with T2D at two timepoints 6 months apart using phenomenological methodology



Data were coded deductively with the HDDF and SST.

- HDDF guided analysis of structural barriers, discrimination, cultural factors, and resilience.
- SST examined four domains of support: emotional, instrumental, informational, and appraisal



Results

Findings underscored the interplay of systemic barriers and interpersonal resources on T2D management

Risk & Structural Barriers

Healthcare Discrimination

Masculinity Norms

Cultural Food Practices

Financial Strain

Protective Factors & Facilitators

Resilience

Faith

Peer Accountability

Workplace openness

Emotional Support

- Care shown through small acts (check-ins, humor) rather than open conversation
- Peer mutual motivation was highly meaningful
- Masculine norms discouraged vulnerability and emotional openness
- Reciprocal peer support bypassed the stigma of "asking for help"

Instrumental Support

- Practical help (meals, transport, meds) was essential for consistent care
- Accepting help often felt like a threat to independence and masculinity
- Peer-based help felt more acceptable than family help due to reciprocity
- Unsolicited family assistance often triggered resistance

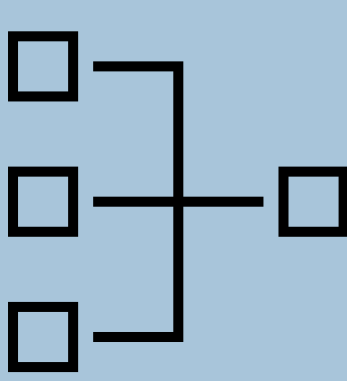
Informational Support

- Informal networks (family, peers, church) were trusted
- Peer knowledge filled gaps left by impersonal clinical encounters
- Conflicting advice required personal judgment to navigate
- Info was most valued when culturally relevant and experience-based

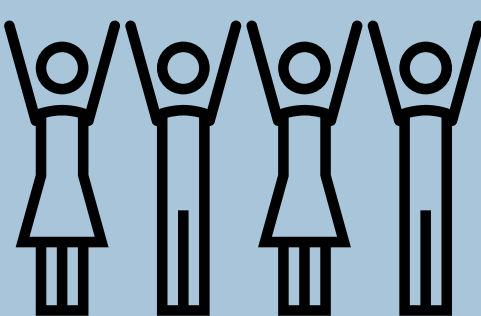
Appraisal Support

- Positive recognition of progress boosted confidence and motivation
- The least recognized type of support, but subtly powerful
- Monitoring by family often felt like criticism rather than care
- Best when affirming effort and respecting autonomy — controlling feedback backfired

Conclusions



Black/African American men with T2D navigate systemic inequities and interpersonal supports through adaptive strategies



Interventions should address structural barriers while strengthening natural support, fostering accountability, and reinforcing culturally grounded approaches to improve self-care management and reduce disparities

